
**DEPARTMENT
POLICY****MA Only**

The medical services authorization data elements are:

- Provider ID number.
- Program Enrollement Type (PET) code.
- Patient-pay amount.
- Begin date.
- End date.

**MANAGED CARE
BENEFICIARY
ENTERS L/H**

It is the local MDHHS's responsibility to enter the medical services authorization. However, see BAM 120 when a beneficiary in managed care becomes an long term care or hospital (L/H) patient.

The Michigan Department of Health and Human Services (MDHHS) and Medical Services Administration (MSA) share responsibility for the medical services authorization for these beneficiaries.

**PET (PROGRAM
ENROLLMENT
TYPES - FORMERLY
LOC CODES)**

Long term care (LTC) facilities and waiver services providers will not be paid unless the appropriate program enrollment type (PET) code is in CHAMPS. For MDHHS staff, adding, removing or changing - PET codes are not negative actions.

Note: Changing a PET code to EXM-DIVM for an L/H or waiver MA patient is a negative action; see BEM 405.

LTC

Long Term Care facilities enter the admission date and other required information directly into the CHAMPS system.

See BAM 120 when a beneficiary with managed care PET codes enters LTC.

Medical Services Administration within MDHHS is responsible for notifying the beneficiary and LTC facility if nursing care is not needed. MSA enters the appropriate PET code and the facility will not be paid by Medicaid.

A beneficiary in an LTC facility may also be enrolled in a hospice. MSA enters PET codes for these cases.

Note: Do not change the post-eligibility patient pay amount when a beneficiary is transferred to a hospital or another LTC facility.

Note: Use instructions in BEM 405 to end the divestment penalty, PET code EXM-DIVM.

Hospitals enter the admission date and other required information directly into the CHAMPS system.

MIChoice waiver agents enter the admission date and other required information directly into the CHAMPS system.

Note: Use PET code EXM-DIVM for a divestment penalty period; see BEM 405.

HOSPICE AND LTC

When a beneficiary receiving hospice care enters LTC determine the patient pay amount (PPA).

When a beneficiary in LTC begins receiving hospice care determine that the PPA has not changed. If it has changed re-enter the correct PPA. PATIENT-PAY AMOUNTS

There are different patient-pay amounts (PPAs):

- **LTC and hospital** - Used to establish Group 2 income eligibility; see BEM 545.
- **Post-eligibility** - Certain L/H patients' share of their cost of care; see BEM 546.

Approval of MA for a month is a positive action even when there is a PPA.

Always enter the PPA when adding MA coverage regardless of:

- How long an application has pended.

- Which month of coverage is being added to an active deductible case.
- Whether the DHS-3227, Tentative Patient-Pay Amount Notice, was sent to the LTC facility.

Adding MA Coverage with a PPA

When adding MA coverage for a month having a hospital, LTC, or post-eligibility PPA:

- The begin date of the PPA is the first day of the month **or** the hospital admission date/LTC admission date, **whichever is later**.
- The end date is the hospital discharge date/LTC discharge date **or** the last day of the month, **whichever is earlier**.

Exception: When MA eligibility will be ongoing for an L/H patient, the end date of the ongoing post-eligibility PPA is 9s.

Changing Post-Eligibility PPAs

When changing a post-eligibility PPA for an MA beneficiary:

- Begin a **higher** PPA the first day of the month following the month in which the negative action pend period ends.
- Begin a **lower** PPA the first day of the month:
 - The change occurred, if it was reported within 10 days.
 - The change was reported, if not reported within 10 days.

Note: Changes that result in a lower PPA include reduced income and higher needs as allowed by BEM 546. For example, a beneficiary will have a higher patient allowance when in an LTC facility only part of a month.

RETROACTIVE ADJUSTMENTS

Do **not** increase or add a PPA for a past period for which the beneficiary already has MA coverage.

Correct PPAs that should have been lower for past periods. In addition to the beneficiary, notify the hospital/LTC facility so that the providers may adjust their MA billings.

LEGAL BASE**MA**

42 CFR 435.725, .726 and .832

42 CFR 456